

STAGE-I PROJECT REPORT

on

Generic Alternative Provider (GAP)

By

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UNDER THE GUIDANCE OF

Prof. Kalyani Pampattiwar

SUBMITTED IN PARTIAL FULFILLMENT FOR THE DEGREE OF
BACHELOR OF ENGINEERING

In

Computer Engineering



DEPARTMENT OF COMPUTER ENGINEERING

SIES GRADUATE SCHOOL OF TECHNOLOGY

NERUL, NAVI MUMBAI – 400706

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Certificate

This is to certify that the project entitled “**Generic Alternative Provider (GAP)**” is a bonafide work carried out by the following students of final year in Computer Engineering

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The report is submitted in partial fulfillment of the degree course of Bachelor of Engineering in Computer Engineering, of University of Mumbai during the academic year 2023 – 2024.

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Project Report Approval

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Declaration

I declare that this written submission represents my ideas in my own words and where others' ideas or words have been included, I have adequately cited and referenced the original sources. I also declare that I have adhered to all principles of academic honesty and integrity and have not misrepresented or fabricated or falsified any idea/data/fact/source in my submission. I understand that any violation of the above will be cause for disciplinary action by the Institute and can also evoke penal action from the sources which have thus not been properly cited or from whom proper permission has not been taken when needed.

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Abstract

In the world of generic drugs, emphasizing their primary benefits, which revolve around their low cost and widespread availability. The absence of significant costs associated with the development and marketing of a novel drug accounts for generic drugs' cost-effectiveness. When a pharmaceutical company's exclusive patent rights to a novel medication expire, it allows other manufacturers to make generic equivalents. This rivalry undoubtedly results in a large drop in the prices of these generic medications. However, a real issue arises since many patients, particularly those with lower incomes, are unaware of these cost-effective choices. To address this issue, the abstract suggests a viable solution in the form of a software program. This software uses modern technologies including Optical Character Recognition (OCR), Entity Extraction, and Cosine Similarity to bring innovation to healthcare. The major purpose of this software is to increase healthcare inclusion and accessibility. It accomplishes this by reading prescriptions, extracting key information, and recommending comparable generic medications. The suggested technological framework not only promises to streamline operations, but also to eliminate errors, thereby improving the efficiency and quality of patient care. The overarching goal is to help modernize healthcare by using these technologies to make vital treatments more accessible and cheaper to a larger population.

Keywords: *Healthcare Accessibility, Cost-Effective Alternatives, Patient-centric care, Affordable Medications, Sustainable Healthcare Ecosystem*

List of Figures

AI	Artificial Intelligence
API	Application Programming Interface
ML	Machine Learning
NER	Named Entity Recognition
OCR	Optical Character Recognition
UI	User Interface

Abbreviations

AI	Artificial Intelligence
API	Application Programming Interface
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Chapter 1

Introduction

1.1 Introduction

hello [1]

1.2 Motivation

1.3 Scope

1.4 Project Schedule

1.5 Document Organization

This report is broken down into chapters. The report begins with Chapter 1, which is the project's introduction, which includes the introduction, the rationale for the solution provided for the problem that was faced, the project's problem statement, and the objectives for each element in the developed framework, as well as the report's structure.

Proceeding to Chapter 2, it presents the Literature Survey and discusses the work in this field. It is further divided into a literature study of previous ways to address, as well as how those solutions were implemented in the real life and the approaches' shortcomings. A contribution to a mini project is also included.

In Chapter 3 of this report, Proposed system, it mentions the importance of having a simple but a rapid system. The proposed system is also explained and how will it overcome the short comings of the existing system. Covering the system design and mention and elaborate the various actors involved in this system with the usage of hardware and software in the suggested system is also covered in this chapter. Finally, the experiment results are there in the last part of this chapter hence concluding this whole chapter of proposed system.

Now what the next phase consits of is covered in Chapter 4 which is planned task for next phase. Everything concludes at Chapter 5 with all the possible roads we can take from our current model and describing each one of the paths that we mentioned under a roof called as future work and after that the report ends with a word of

conclusion that concludes our report and gives an overview of everything.

Chapter 2

Literature Survey

2.1 Survey of Existing system

2.2 Research Gaps

2.3 Problem Definition

2.4 Objectives

1. To address the lack of awareness about generic drug alternatives, initiate educational campaigns to inform individuals about their availability and effectiveness, while debunking misconceptions.
2. To streamline prescription management and facilitate insurance claims, create and maintain a centralized database, offering users easy access to their medical history and simplifying record-keeping.
3. To contribute to medical knowledge, launch research initiatives to study patient outcomes during the transition from brand-name to generic medications, providing evidence-based insights for better prescription practices.
4. To address the need for cost-effective healthcare, quantify and demonstrate potential cost savings through the use of generic drugs. This encourages their adoption, benefiting both individual patients and healthcare organizations.

Chapter 3

Proposed System

3.1 Present Report on Investigation

3.2 Component of the System

Chapter 4

Result and Discussions

4.1 Implementation

4.2 Results of Proposed System

Chapter 5

Conclusion and Future Work

5.1 Conclusion

5.2 Future Scope

Bibliography

- [1] Gustavo Saposnik, Muhammad M Mamdani, Maria Alicia Terzaghi, Maria L. Saladino, Berenice A Silva, Philippe N. Tobler, and Fernando J. Cáceres. The role of prescribing generic (non-proprietary) drugs in the prevalence of therapeutic inertia in multiple sclerosis care. *Frontiers in Neurology*, 9, 2018.